

Men's Health Clinical Reference

Provider Source of Truth

Testosterone · KORB Rise · Hair Loss · KORB Health Group · Internal Use Only

This is the single source of truth for the KORB Men's Health program. It supersedes the Men's Health Program Document dated 2/10/2026 and any prior Testosterone SOP or Appendix. It supports provider decision-making and does not replace clinical judgment. Specific Tebra favorite fields and visit/refill dates are generated by the TRT Provider Tool, not printed here.

Scope and How to Use This Document

Men's Health covers three areas: **Testosterone Replacement Therapy** (the base program), plus **KORB Rise** and **Hair Loss** as add-ons. Sermorelin is no longer part of Men's Health; existing Sermorelin patients transfer to Functional Health & Longevity.

This document states the rules. The TRT Provider Tool generates the specific numbers: exact injection volume, correct days supply, visit and refill dates from the PMP last fill, the Tebra favorite fields, and copy-paste note text. Do not maintain a second copy of the Tebra favorites anywhere else. If the rules here and the tool ever disagree, stop and escalate rather than guessing.

Identifying the Appropriate Candidate

Candidates meet **both** clinical and laboratory criteria. Labs alone are not sufficient, and symptoms alone are not sufficient.

Symptoms: fatigue or low energy, reduced libido or sexual dysfunction, decreased muscle mass or strength, increased fat mass, mood changes or low motivation, cognitive complaints, reduced exercise tolerance or recovery.

Biochemical evidence of low or inappropriately low testosterone, interpreted in clinical context.

Symptoms meaningfully affecting quality of life, daily function, or overall health.

Total testosterone below 300 ng/dL is the commonly used threshold to define deficiency (AUA). The Endocrine Society uses 264 ng/dL. Symptoms may occur even when values fall inside population reference ranges.

Contraindications and When to Refer

Do not initiate testosterone therapy in men with any of the following. Refer to PCP, urology, or the appropriate specialist.

Known or suspected prostate or breast cancer

Uncontrolled polycythemia, or elevated hematocrit at baseline

Severe untreated obstructive sleep apnea

Uncontrolled heart failure; MI or stroke within the last 6 months

Thrombophilia

Active desire for fertility in the near term (relative contraindication; counsel and consider referral)

PSA above 4.0 ng/mL, or above 3.0 ng/mL with high prostate cancer risk, without urological evaluation first

Pharmacy Routing by State

Testosterone is offered in **Texas and California only**. There are no other states.

State	Pharmacy	Notes
Texas	Premier Pharmacy	Bill to office / ship to patient. Supplies not itemized on the favorite.
California	Empower Pharmacy	Bill to office / ship to patient. Syringes and needles itemized on the favorite.

Only named providers may prescribe testosterone, and only in their state.

Texas: Don Stevenson only.

California: Larisa and LaTonya only.

Prescribing authority follows the provider's registration in the state where the patient is located. If you are not named above for that state, do not prescribe. Route the patient to a provider who can.

Testosterone is a Schedule III controlled substance. Confirm the patient's state of residence before prescribing, and confirm the dispense length is permitted in that state before sending a fill longer than 90 days.

Dose Ladder

Testosterone cypionate 200 mg/mL, dispensed as a 10 mL vial (2000 mg). Four standardized weekly doses. Every step is exact on all three routes with no rounding, so the weekly dose in the chart is the weekly dose the patient receives regardless of route.

Weekly dose	IM once weekly	SQ twice weekly	SQ three times weekly	Days supply
96 mg	0.48 mL	0.24 mL	0.16 mL	145 days
120 mg	0.60 mL	0.30 mL	0.20 mL	116 days
144 mg	0.72 mL	0.36 mL	0.24 mL	97 days

Weekly dose	IM once weekly	SQ twice weekly	SQ three times weekly	Days supply
168 mg	0.84 mL	0.42 mL	0.28 mL	83 days

Start at 96 mg or 120 mg per week. Typical starting range in practice is 100 to 150 mg per week. Starting at the top of that range is a common cause of elevated estradiol, elevated hematocrit, and supraphysiologic levels. Titrate up on labs and symptoms, not on request.

Syringes and Injection Technique

Testosterone cypionate is a viscous oil. It requires a **Luer lock** syringe with a separate draw-up needle and injecting needle. **Do not use insulin syringes.** They have a fixed needle that will not draw the oil reliably, and they are marked in units, which do not appear on a Luer lock barrel.

Route	Syringe	Draw up with	Inject with
SQ (twice or three times weekly)	1 mL Luer lock	23G 1 inch	27G ½ inch
IM (once weekly)	3 mL Luer lock	23G 1 inch	25G 1 inch

SQ sites: abdomen, upper thigh, or gluteal region, 45 to 90 degrees. IM sites: gluteal or deltoid. Rotate sites every injection. Counsel on sharps disposal per state and local rules.

Route Selection

SQ twice weekly is the default starting route. It gives the most stable levels with a reasonable injection burden, and is the standard starting frequency in most modern TRT practice.

IM once weekly is an acceptable starting alternative for patients who prefer fewer injections. Expect greater peak-to-trough variation.

SQ three times weekly is second-line. Do not start patients here.

When to move to SQ three times weekly. Use it as an adjustment for a patient already on twice weekly who has: low SHBG, trough symptoms in the day or two before the next dose, rising estradiol with symptoms, or rising hematocrit. Increasing frequency lowers the peak, which reduces both aromatization to estradiol and the erythrocytosis stimulus. Raise frequency before reaching for an aromatase inhibitor. Document the indication in the note.

Titration Targets and Thresholds

These are the written rules for moving up or down the ladder. They mirror how the Functional Health & Longevity program uses an IGF-1 target band. **Draw testosterone at trough**, on an injection day before the dose is given. A peak draw reads high and hides a symptomatic trough.

Marker	Target / Action Threshold	Action
Total testosterone (trough)	Goal band 400 – 700 ng/dL	In band with symptoms resolved: hold current dose.
Total testosterone (trough)	Below 400 ng/dL with persistent symptoms	Move up one step on the ladder. Recheck in 6 to 8 weeks.
Total testosterone (trough)	Above 700 ng/dL	Move down one step, even if the patient feels well.
Total testosterone (trough)	Above 1000 ng/dL	Reduce dose regardless of symptoms. Supraphysiologic levels carry cardiovascular and polycythemia risk.
Hematocrit	50 – 53%	Warning range. Recheck, review sleep apnea and hydration, consider increasing injection frequency.
Hematocrit	Above 54%	Intervene. Hold or reduce dose, increase frequency, and discuss therapeutic phlebotomy. Do not continue unchanged.
Estradiol	Elevated with symptoms (gynecomastia, nipple tenderness, mood, fluid retention)	Increase injection frequency first. Reduce dose if that is insufficient. An aromatase inhibitor is a last resort.
PSA	Rise greater than 1.4 ng/mL within 12 months, or absolute above 4.0 ng/mL	Refer to urology before continuing therapy.

Change one variable at a time. Adjusting dose and frequency together makes it impossible to know which one worked. Recheck labs 6 to 8 weeks after any change, drawn at trough.

Days Supply and Refill Timing

Days supply depends on the dose, but nothing is written past 90 days. The ladder table above shows how long a 10 mL vial physically lasts at each dose. Two of those figures exceed 90 days, and the prescription is capped there. The days supply also goes in the sig line so the pharmacy sees it on the label, not just in the field.

Weekly dose	Vial physically lasts	Days written on Rx	Discard remainder?
96 mg	145 days	90	Yes, at 90 days
120 mg	116 days	90	Yes, at 90 days
144 mg	97 days	90	Yes, at 90 days
168 mg	83 days	83	No, vial is empty

When the cap applies, the whole cycle runs on 90 days, not on the vial. Refill release, follow-up visit, and labs are all calculated from 90 days after the last fill. Counsel the patient to discard any remaining medication when the new supply arrives. Do not tell a patient their vial lasts longer than 90 days, and keep beyond-use reasoning out of patient-facing material entirely. The patient needs one rule: discard at 90 days.

Refill timing is driven by the **PMP last fill date**, not by the visit date:

Pharmacy can release = PMP last fill date + days supply. Premier will not release earlier on a controlled substance.

Send the Rx 4 days before that release date. Sending earlier is what produces the too-early callback.

See the patient on or before the Rx send date, so therapy can be reauthorized before the fill goes in.

Order labs 14 days before the visit, future-dated in Tebra. Results can take up to a week, so the patient has roughly a one-week window to get drawn. Tell them not to leave it to the final days.

The TRT Provider Tool calculates all four dates from the exam date and the PMP last fill date. Check the PMP at every visit and enter the actual last fill date, not the last date you wrote a prescription. Tell the patient the pharmacy cannot release before the release date and that shipping is on top of that.

Laboratory Monitoring

Standard TRT panel, ordered at baseline and at each follow-up. No add-ons within Men's Health.

Test	Quest code
FSH & LH	7137
PSA, Total	5363
Prolactin	746
CBC	1759
Estradiol	4021
Testosterone, Total, MS	15983

Draw testosterone at **trough**, on an injection day before the dose.

Order labs future-dated for 14 days before the next visit, and post the handoff to the lab queue so Ops can schedule the patient reminder.

Patients needing a broader workup are referred to Functional Health & Longevity rather than adding tests here.

Add-Ons

Men's Health patients may add **KORB Rise** (sexual health), **Hair Loss** treatment, or **Skin Care**. These are cross-program offerings available to Weight Loss, Men's Health, Women's Health, and Functional Health & Longevity patients alike.

Clinical detail for all three add-ons lives in the Add-On Clinical Reference, not here. That includes formulations, dosing, contraindications, pharmacy routing, charge codes, and Tebra favorites. It is maintained as a single document so the same product is never described in two places. Use the Add-On Selector tool to confirm what is appropriate for a given patient's program, sex, and age.

Two items are called out here because they intersect directly with testosterone therapy:

KORB Rise and nitrates. Absolute contraindication. A patient on nitrates for chest pain must not receive KORB Rise. Ask directly at every visit rather than relying on the medication list.

Finasteride and testosterone. Finasteride is offered for male pattern hair loss and blocks conversion of testosterone to DHT. Counsel on the sexual side effect profile, since it overlaps with the symptoms testosterone therapy is being used to treat and can confound your read on whether TRT is working.

Referral to Functional Health & Longevity

Sermorelin and the other peptides now sit in Functional Health & Longevity. Refer a Men's Health patient there when the goal is growth-hormone axis support, recovery, body composition, or broader longevity work rather than androgen replacement.

Programs can run in parallel. A patient may be enrolled in Men's Health and Functional Health & Longevity at the same time, or in Men's Health alongside Weight Loss. This is not an either/or referral. Confirm the combined plan is clinically appropriate, reconcile the full medication list across both programs, and make sure the patient understands which visits and labs belong to which program.

Pricing and Charge Codes

Item	Price	Code
Initial TRT labs	\$99	FITTRTQ100
Quarterly TRT (visit, medication, shipping)	\$399	FITTRTQ001

Add-on pricing and charge codes are held in the Add-On Clinical Reference so they are maintained in one place. KORB does not accept insurance. Compounded medications are not FDA-approved for these specific uses.

Open Items

Flagged and not yet resolved. Do not treat these as settled:

Lab panel review with Dr. Rose. Current guidelines position LH, FSH, and prolactin as baseline diagnostic tests rather than routine quarterly monitoring, and SHBG and free testosterone are not in the standard panel despite driving dose decisions. Panel rework deferred.

Draw-up needle gauge. This document specifies 23G to match existing favorites. A 25G has also been proposed. The wider 23G draws viscous oil faster. Confirm before the favorites are rebuilt.

Schedule III dispense length. Confirm TX and CA permit a 145-day or 116-day fill before the two lower doses go live.

PERFORM (FarmaKeio ODT). A second sexual health product exists in the prior protocol. Confirm whether it is still offered.

korbhealth.com is out of date on KORB Rise ingredients, still lists Sermorelin under Men's Health, shows hair loss starting at \$99 without noting topicals are \$149, and carries a peptide explainer block that no longer applies to any product on the page.